Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bloods / symptom review	FBC, renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
ECG / echo	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / DXA	Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
Early-breast surveillance	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine CT / markers if asymptomatic
Advanced-disease response	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Before-surgery pathway only	Confirm exact NCCP neoadjuvant eligibility	Agree clinical response / surgery reviews	Assess response during 4–8 months	Inadequate response: surgery / alternative review	Proceed to agreed operation / next treatment

THE WHOLE TREATMENT JOURNEY

Letrozole · daily hormone therapy: the whole treatment pathway

Postmenopausal hormone-sensitive breast cancer; setting-specific pathways

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Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Early: surgery / chemo ESMO_EBC2024	Surgery / pathology guide the plan	Any indicated chemotherapy first	Start endocrine after chemotherapy	Review recovery and lasting effects	Agree next endocrine decision
Early: radiotherapy ESMO_EBC2024 · EVIQ_RT3650	Decide if radiation is indicated	Usually after surgery and any chemo	AI may overlap; tamoxifen timing agreed	Complete RT / review local effects	Continue endocrine beyond RT if planned
Early: endocrine medicine NCCP 00371	Confirm setting and prior treatment	Start daily tablets at agreed point	Continuous medicine between reviews	Review planned total course / extension	A review is not an automatic stop
Before surgery: letrozole NCCP 00371	Confirm HR-positive / HER2-negative criteria	Start daily preoperative treatment	Response review during 4–8 months	Plan surgery; earlier if inadequate response	Pathology guides next treatment
Advanced: medicine NCCP 00371	Confirm exact advanced indication	Daily tablets	Continue if beneficial	Review cancer control and tolerance	Agree next treatment / care
Advanced: local treatment ESMO_MBC2023	Define any local-control / symptom need	Surgery / radiation only if indicated	Teams agree overlap or sequence	Review symptoms and local effects	Reassess goals if disease changes
Safety checks NCCP 00371 · SMPC	Baseline FBC, renal and liver profile	Review symptoms / interactions	Bloods when clinically indicated	Risk-based cardiac / other checks	Follow unresolved abnormalities
Scans / bone health NICE_NG101 · ESMO_EBC2024 · ESMO_MBC2023	Baseline disease and fracture assessment	Confirm which setting-specific rows apply	Keep agreed scan / DXA appointments	Early: mammography; advanced: response scans	Name who books and reviews results

What each check is for

1 Bloods and symptoms

Baseline FBC, renal and liver profile; later blood tests are clinically indicated, not automatic chemotherapy-cycle tests. Review symptoms, adherence, medicines and relevant menopause uncertainty.

Sources: NCCP 00371

2 Heart monitoring

There is no routine ECG or echo schedule for letrozole alone. Assess cardiovascular risk factors and new symptoms as clinically indicated.

Sources: NCCP 00371 · SMPC

3 Bone health

Adjuvant AI: offer baseline DXA unless already receiving adjuvant bisphosphonates. T-score above −2.0 with no additional risk factors: reassess risk / BMD every 1–2 years. On bone-protective treatment: generally about every 2 years. Fracture, substantial bone loss or changed risk may need earlier review; the team individualises the plan.

Sources: NICE_NG101 · AIBL2025

4 Cancer scans

Early / curative: annual mammography of remaining breast tissue; no routine mammography on the mastectomy side, CT, bone scans or tumour markers if asymptomatic. Advanced: response assessment generally every 2–4 months, adapted to disease pace and symptoms; less often may suit indolent disease. The team selects imaging, not automatically CT chest/abdomen/pelvis. New symptoms can bring assessment forward.

Sources: ESMO_EBC2024 · ESMO_MBC2023

5 Markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

Sources: ABC5_MARKERS

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending